

Procedure Assessment for Resident – Intestinal anastomosis

Resident Resident year ..2..... Operation

Date Resident Staff

	F – Fails M – Meets expectation E - Exceeds	F – Fails M – Meets expectation E - Exceeds	Staff comments
Preparation :			
- Short brief an information : Hx, PE, Lab, X-ray	F M E	F M E	
- Time out	F M E	F M E	
1. Antibiotics prophylaxis			
2. What are the critical or non-routine steps?			
3. How long will the case take?			
4. What is the anticipated blood loss?			
Operation :			
Incision and approach	F M E	F M E	
Relevant anatomy : identify vessels	F M E	F M E	
Anticipates steps in procedure	F M E	F M E	
1. Adequate mobilization should be achieved so the intestines can stay together without tension			
2. Appropriate mesenteric devascularization (less than 1 cm from the end of intestine) with clear visualization of all intestinal layer	F M E	F M E	
3. Both side of intestine appears healthy without sign of ischemia and tension before starting anastomosis	F M E	F M E	
4. Careful place intestinal clamp gently on both side of intestine and place together in correct fashion (each mesentery is on the same side) with open end pointing upward	F M E	F M E	
5. Start stitching a posterior Lambert (seromuscular layer) stitch with a non absorbable suture 3/0 or 4/0 : each stitch should be place about 5 – 10 mm from each other (an intestinal flipping for double anterior stitching technique is not allowed in this evaluation)	F M E	F M E	
6. Continue with posterior over and over stitch with absorbable suture 3/0 or 4/0 each bite should be place about 3 – 5 mm from each other: all layer must be included in each bite	F M E	F M E	
7. An intestinal clamp should be release, readjust and reapply in order to minimize a tension before perform anterior layer stitching	F M E	F M E	
8. An anterior Cushing or Cornell mucosal inversion stitching technique might be performed for an anterior layer (other technique could be utilized at this point)	F M E	F M E	

9. After the completion of previous step, both intestinal clamps must immediately be released in order to minimized an ischemic process	F	M	E	F	M	E	
10. Anterior Lambert stitch is performed with non absorbable suture until the anastomosis is completed	F	M	E	F	M	E	
11. The mesenteric defect is carefully close in order to avoid any bleeding and vascular injury	F	M	E	F	M	E	
12. The patency of lumen is properly checked with imbrication technique (pinching doughnut technique)	F	M	E	F	M	E	
13. All procedure should be completed in proper time length (less than 1 hr.)	F	M	E	F	M	E	
Knowledge of critical points of operation	F	M	E	F	M	E	
Demonstrates proper handling of tissue and instrument	F	M	E	F	M	E	
Good basic skills : suture/knot tying	F	M	E	F	M	E	
Follows a logical sequence in case	F	M	E	F	M	E	
Responds appropriately to acute events	F	M	E	F	M	E	
Follow-Up : Completed Sign-out : Nurse verbally confirms (procedure, instrument, specimen, any problems)	F	M	E	F	M	E	
Complete post-operative orders	F	M	E	F	M	E	
Saw patient in recovery room	F	M	E	F	M	E	

Resident self assessment : What did you learn from this case : _____

Staff assessment : What would you recommend to help the resident improve their skills/ preparation : _____

Overall assessment : F M E

Signature (Staff)